



**National Health Service
Standards 2nd Edition
for Papua New Guinea
Volume Two**

**National Quality Standards and
Accreditation Program for Health
Services in Papua New Guinea
2021-2030**

2021

National Department of Health
National Health Services Standards 2nd Edition Volume Two
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NATIONAL HEALTH SERVICE STANDARDS VOLUME 2
NATIONAL QUALITY STANDARDS AND ACCREDITATION PROGRAM

Applicable to: All Staff

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Version Control and Document History

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2011	Initial National Health Services Standards Vol 1-2-3	V1
2021	Modified - NHSS 2 nd Edition Vol 1-2	V2

ACRONYMS

Acronym	Full term	Definition
BPR	Business process re-engineering	A method for redesigning the way work is done to better support the organization's mission and reduce costs.
CQI	Continuous quality improvement	The process of continuously improving quality, e.g. we are working to reduce surgical wound infection rates below our own best performance, and then below the best in our Region and then the best in the nation etc.....
CSF	Critical success factor	Something that an organisation must do within a <i>key result area</i> , e.g. timely emergency care, high quality maternity care, timely elective surgery
DALY	Disability adjusted life year	A year of life adjusted for illness or disability, e.g. a year of life with a severe disability or illness is of less value than a year free from disability or illness. It is used for evaluating the value for money of health care interventions.
DOTS	Directly Observed Therapy, Short-course	A major initiative in the WHO international TB eradication program
FBO	Faith-based organisation	
GoPNG	Government of PNG	
IPC	Infection prevention control	
KM	Knowledge Management	A system for providing current, evidence-based information, especially to clinicians to aid clinical decision-making
KPI	Key performance indicator	A measure that indicates whether a <i>critical success factor</i> is being successfully achieved, e.g. wound infection rate, immunisation rate, maternal mortality rate
KRA	Key result area	One of a small number of things that are important elements of the role of an organisation, e.g. emergency care, elective surgery, outpatient care
NDoH	National Department of Health	
NGO	Non-government organisation	
NHP	National Health Plan 2011-20	
PDSA	Plan, do, study, act	"The quality cycle"
Preceptor		A senior accreditation Surveyor elected by the Surveyors to provide guidance in development of the accreditation process and in accreditation surveys
QA	Quality assurance	The process of ensuring that a specified, minimum level of quality is achieved, e.g. no-one waits longer than 30 minutes to be assessed in an emergency department.
QALY	Quality adjusted life year	A year of life adjusted for the quality of life of an individual, e.g. a year of life with a severe disability or illness is of less value than a year free from disability or illness. It is used for evaluating the value for money of health care interventions.
QC	Quality control	The process of rejecting faulty products. It has been largely superseded by CQI and has little application in contemporary health care except perhaps in rejecting faulty products such as instrument trays, sterile packs, pharmaceuticals

RCA	Root cause analysis	A technique for identifying all the contributing factors to an adverse event
RCT	Randomised controlled trial	A technique for comparing the effectiveness of two courses of action
TQM	Total Quality Management	A philosophy of quality management which has been superseded by CQI
NRSTTH	Level 6 National Hospital	National Referral Specialist Teaching Training Hospital
WHO	World Health Organisation	The international body, affiliated with the United Nations, that sets many standards, objectives and processes for improving health, especially in developing countries

EXECUTIVE SUMMARY

The National Health Service standards (NHSS) are essential to ensure that all health services are integrated and clearly articulated and can be planned, managed, monitored and evaluated in a strategic manner. They allow for health services planning at all levels and across all health programs and service types, inform financial, workforce, information and infrastructure requirements, clarify governance arrangements, and assist with evaluating ongoing care to ensure safe, quality health care delivery to meet optimal outcomes.

National Health Services Standards and Accreditation Program Volume Two (2) provides a framework, commitment and process for the implementation of Papua New Guinea (PNG) healthcare standards to achieve continuous improvement, ‘to strengthen safe, quality health care by continuously advancing standards and education nationally.

What is accreditation?

“Accreditation is a formal process to ensure delivery of safe, high quality health care based on National Quality Standards and processes devised and developed by health care professionals for health care services. It is public recognition of achievement by a health care organization, of meeting all of the requirements of the National Health Service Standards” (NHSS).

In PNG, healthcare accreditation is a peer review of the quality of care and services. It has international recognition to assess and improve the quality, efficiency, and effectiveness of healthcare organisations. It is also a way to publicly recognise that the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), Provincial Health Authorities (PHAs), Non-government Health Organisations (NGOs) and Private Sector Health Organisations, have met the NHSS National Quality Standards.

In the past, this process was voluntary and recently only a small number of hospitals have undertaken a voluntary accreditation process.

From July’2021 onwards, the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), all PHAs (and other Health Service Organisations) will be required to undertake a four (4) year quality assessment and improvement program that supports excellence in integrated health service delivery models and approaches, patient care services and public health programs.

The existing National Quality Standards (NQS) have been updated and redesigned to ensure the provision of safe, quality care services, effective public health interventions and the achievement of continuous quality improvement in terms of integrated health services delivery. If all of this is achieved, accreditation follows.

This approach benefits our health care system to now plan for a cycle of accreditation of safe quality health care the National Referral Specialist Tertiary and Teaching Hospital, all Provincial Health Authorities, and other Health Service Organisations.

Adoption of these updated National Quality Standards by a health service organization shows its willingness to have its compliance with the NQS assessed by a team of trained, experienced and practicing health care professionals – i.e. ‘peer surveyors’.

The revised National Quality Standards (NQS) are aligned with the National Health Plan (NHP) 2021-2030 and strengthened to enable all PHAs, and other health service organisations, to strive to achieve an individual continuous quality improvement program and undertake a formal accreditation process.

The revised National Quality Standards are aligned with the National Health Plan (NHP) 2021-2030 and strengthened to enable the National Referral Specialist Tertiary and Teaching Hospital, all Provincial Health Authorities and other Health Service Organisations, to strive to achieve a continuous quality program and undertake an accreditation process.

A formal accreditation recognition for the NRSTTH, PHAs and ‘other health service organisations’ gives the public confidence that a health service is meeting the National Quality Standards. Accreditation is awarded jointly by the Minister for Health and the Secretary of the National Department of Health.

The NRSTTH, all PHAs and other ‘Health Service Organisations’ will undergo a program of quality improvement and accreditation with self-assessment surveys, periodic reviews and a PHA organizational wide accreditation survey, meeting compliance to the National Quality Standards and particularly those identified as mandatory standards.

The National Department of Health (NDoH), also reserves the right to inspect health services which do not have current accreditation to ensure that they are achieving a satisfactory degree of compliance with the National Quality Standards.

BACKGROUND

Hospital standards were established in Papua New Guinea in 1997 to promote the best possible quality of health care and continuous quality improvement of public health services. This was crucial to the mission of serving “*the rural majority and the urban poor*”. The further development of the standards and progress towards achieving them by all hospitals was a major contribution to the development of health services.

In 2010, with the release of the National Health Plan 2011-2020, these National Quality Standards were amended to make them suitable for application in all health services, i.e. Provincial Health Authorities, Church Health Services, Referral Hospitals, Regional Hospitals, Provincial Hospitals, District Hospitals, Health Centres and Community Health Posts.

The National Health Service Standards (NHSS) outline the minimum requirements to be met to ensure safe, appropriate and quality health services delivery. These National Health Service Standards have three major purposes:

- i. To provide a standard set of criteria for ‘Levels of Care’, and ‘Essential Health Services’ supported by the core components for health system strengthening that is accessible, equitable, affordable efficient, clinically viable and integrated with evidenced based public health interventions;
- ii. To provide a quality improvement program with a set of National Quality Standards and an Accreditation process to guide health services through self-assessment, periodic review and a 4year cycle of compulsory accreditation to meet Papua New Guinea National Quality Standards; and
- iii. To provide standards for planning, design, development, commissioning, operation, maintenance and disposal of health facilities infrastructure and medical technology for the public sector in PNG.

The 2011 National Quality Standards for Health Services in Papua New Guinea – Health Service Accreditation Program User Guide continues to be utilized to assist the NRSTTH, all PHAs and other ‘Health Service Organisations’ in the use of the National Quality Standards for Health Services in Papua New Guinea and in the accreditation process.

It is important that all stakeholders have a shared understanding of the National Quality Standards that is expected for the NRSTTH, all PHAs and other ‘Health Service Organisations’ to achieve in accessibility, safety and effectiveness. The National Quality Standards for Health Services in Papua New Guinea help achieve this shared understanding.

The updated National Quality Standards (NQS) step from the National Health Plan’s 2021-2030 Key Result Areas and from many other authoritative sources.

National Quality Standards (NQS) are defined as documents that provide requirements, specifications, guidelines, or characteristics that can be used consistently to ensure that materials, products, processes, and services are fit for their purpose.

National Health National Quality Standards primary aims are: to protect the public from harm and improve the quality of health service provision.

Standards provide organizations with the shared vision, understanding, procedures, and vocabulary needed to meet the expectations of their stakeholders. Because standards present precise descriptions and terminology, they offer an objective and authoritative basis for organizations and consumers around the world to communicate and conduct business.

The National Quality Standards (NQS) reflect the contemporary ideals for Boards of Governance for the NRSTTH, PHAs and other 'Health Service Organisations', and their Senior Executives, with regards to leading and managing health services.

As much as possible they focus on the core business of public health and patient care and are based in evidence about what works best. They serve as minimum National Quality Standards to underpin safe health service delivery and they also promote continuous quality improvement.

They do not set out in detail all the standards to which health services should aspire but, by referring to other, more detailed standards; they aim to promote high quality in all that a health service does i.e. infection control guidelines, treatment manuals, etc.

Accreditation is one important way of monitoring and promoting compliance with the National Quality Standards that must be achieved in the NRSTTH, all PHAs and other 'Health Service Organisations'. It draws on the skills of experienced Health Services Managers and Clinicians to assess compliance of health services against the National Quality Standards for Health Services in Papua New Guinea and it is a valuable process of peer review and networking.

Accreditation is a high-level process which tests policies, processes and systems to ensure that they are working and supporting compliance with standards. It also encourages health services to monitor their own compliance with the National Quality Standards.

PURPOSE OF THE NATIONAL QUALITY STANDARDS FOR HEALTH SERVICES IN PAPUA NEW GUINEA

It must be the objective of the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), all Provincial Health Authorities (PHAs) and other 'Health Service Organisations' who provide public health, clinical and primary health care services, or who govern or manage them, to achieve safe, high quality outcomes for their patients and their communities.

The NRSTTH, all PHAs, and other 'Health Service Organisations' must also be efficient, effective, accessible, appropriate and acceptable to those who use them. All these requirements are set out in more detail in the National Quality Standards (NQS) for Health Services in Papua New Guinea.

The NQS are flexible enough to be applied across the NRSTTH, all PHAs, and other 'Health Service Organisations' and can apply to hospitals, urban and rural health services, public health programs, Church-run health services, and non-government and private sector health organizations.

Small health services are not expected to provide the same range of services as the NRSTTH, a PHA Provincial Hospital or a large District Hospital but, where it does provide a service, it must meet the nationally prescribed standard for quality and safety.

This Volume 2, 2nd edition of the National Quality Standards will continually be refined and developed to establish a minimum standard of safety and quality, and to also promote ongoing continuous improvement in ALL PHAs and other health services organisations.

ROLE OF ACCREDITATION IN ACHIEVING THE NATIONAL QUALITY STANDARDS

National Quality Standards Accreditation is intended to:

- Promote a uniformly high level of compliance with the National Quality Standards for Health Services in Papua New Guinea;
- Promote continuous improvement of the performance of the NRSTTH, all PHAs and other 'Health Service Organisations'; and
- Build a national consensus about what is good practice.

The National Quality Standards and the accreditation process can be applied to any health service, i.e. National Referral Specialist Tertiary and Teaching Hospital, and within a Provincial Health Authority, Church Health Services, NGOs, Provincial Hospitals, Rural (District) Hospitals, Rural; and Urban Health Centres, Community Health Posts and (Village) Health Posts.

National Quality Standards either specify, at a high level, what should be done or they refer to other standards, i.e. Standard Treatment Manuals, Clinical Standards, legislation, regulations, policies and guidelines.

They also support and promote reporting to, and monitoring by, other authorities such as the National Department of Health (NDoH).

The National Quality Standards Accreditation process involves a team of trained '*Peer Surveyors*' from outside the NRSTTH, individual PHA, or other 'Health Service Organization, being reviewed, who visit a number of health services at different levels of care, to assess its compliance with the National Quality Standards for Health Services in Papua New Guinea.

The Surveyors rate the health service as being either:

- fully compliant;
- partially compliant; or
- not compliant

with each of the National Quality Standards and write recommendations for achieving improved compliance.

Standards accreditation is now compulsory and each Board of Governance will be held accountable by the Minister for Health and Secretary of Health, for implementation of the process within their respective PHA province.

‘Compulsory Accreditation’, will now be, a key performance criteria, of ALL Boards of Governance, reflected in their agreements between the Minister for Health and the National Health Board.

The relevant Chief Executive Officer (CEO) and the Executive Management team of the NRSTTH, individual PHA, and other ‘Health Service Organisation’ will also have within their employment performance contract, a performance criterion on achievement and/or working towards formal accreditation and evidence of compliance to National Health Standards.

Each PHA and the NRSTTH will select from within each level of care across their districts and include health services and health facilities (government, church health service providers and non-government organisations), priority public health programs and partnering non-government organisations that will be assessed against the National Quality Standards for Accreditation.

NRSTTH - PHA health service accreditation is a process of peer review, with Surveyors and the staff of the health service learning from each other.

NRSTTH - PHA internal accreditation surveys (self-assessment or peer review) also help to identify activities that are not adequately resourced or do not meet Standards but ensuring that the health service is doing the best it can with the available resources.

The objective of the peer survey team is to support the NRSTTH, individual PHA health, or other ‘Health Service Organisation’ in improving its performance; not to punish shortfalls in achievement.

Organization-wide National Quality Standards accreditation surveys are carried out every four 4 years within the NRSTTH, all PHAs, and other ‘Health Service Organisations’.

Accreditation Surveyors are trained professionals, experts in interpreting the National Quality Standards and in their applying of them across different PHA health care settings including the NRSTTH.

SCOPE OF NATIONAL QUALITY STANDARDS AND ACCREDITATION FOR HEALTH SERVICES

The key components of the National Quality Standards (NQS) & Accreditation are:

- National Quality Standards that the NRSTTH, an individual PHA (including church health services, NGOs and private sector organisations) work to achieve,
- Annual self-assessment to evaluate performance against set NQS,
- NDOH, NRSTTH and across PHAs Peer Surveyors assistance and guidance with organisations self-assessment,
- Peer Surveyors (a team of accreditation trained surveyors) to provide an independent assessment of the NRSTTH, individual PHA, and other 'Health Service Organisations' performance against the NQS,
- Quality improvement process undertaken by the NRSTTH, Individual PHA, and/or other 'Health Service Organisation' to address the recommendations from the onsite surveys,
- Accreditation based on Survey Report and approval of National Health Board (NHB).

The standards comprise a series of standards, criteria and elements, arranged under graded ratings that reflect increasing maturity of the NRSTTH, individual PHA, and/or other 'Health Service Organisation' and its quality improvement activities.

NHSS ACCREDITATION PRINCIPLES

NRSTTH, PHA and other 'Health Service Organisation' accreditation principles support 'best practice' designed to facilitate a culture of continuous quality improvement.

The following principles can be applied to all aspects of health service within the NRSTTH, an individual PHA, and other 'Health Service Organisation'.

A focus on patient and people centred care is demonstrated by:	• understanding the needs and expectations of current and future patients and communities, • ensuring patients are the priority, • evaluating the service from the patient and community perspective.
Effective leadership is demonstrated responsibility and commitment to safe quality healthcare service delivery, continuity quality improvement and performance by:	• providing clear direction and information for the NRSTTH, and individual PHA, or other 'Health Service Organisation', • pursuing the ongoing development of strategies, systems and methods for achieving safe quality health service delivery, • inspiring and motivating the health workforce and encouraging employees to contribute, develop and continually learn, • using data and evidence to inform practice and manage change, • Considering health projects that are new, innovative and creative.
Continuous quality improvement is demonstrated by management and staff by how they continually strive to improve the quality of care. Continuous quality improvement assists the NRSTTH, individual PHA, or other 'Health Service Organisation' through:	• looking for practical ways to improve as an essential part of everyday practice, • consistently achieving and maintaining quality care that meets patient and community needs, • monitoring health outcomes in patient care to learn and improve both the care and its future results.

Evidence of outcomes is demonstrated by the NRSTTH, individual PHAs and other 'Health Service Organisations' using health data and information for ongoing the measurement and analysis of performance. Indicators of good care processes or, wherever possible, outcomes of care, demonstrate a commitment to maintaining quality and striving for ongoing improvement by:	• collecting critical data and information about key processes, outputs and results, • reflecting on actions on those factors that lead to improved health and/or quality of life for patients, broader communities or to improve operational performance.
Striving for best practice is demonstrated by the NRSTTH, individual PHAs, and other 'Health Service Organisations' comparing their individual performance with others and by ensuring learnings from others to demonstrate through:	• discovering new techniques and technologies, and using them to achieve improved performance, • increasing the efficiency and effectiveness of systems and processes, • improving patient and community satisfaction and outcomes.

NATIONAL QUALITY STANDARDS & ACCREDITATION (NQS&A) FRAMEWORK

The NQS&A focuses on the issues considered to be of critical importance in providing safe, quality health care.

The standards have been updated to reflect the NHP 2021-2030 “**Health and Wellbeing will be enjoyed by All**”, and essential principles and values of PNG healthcare system, Key Results Areas of engaging with communities, working together in partnerships, universal health coverage, targeted priorities and strengthened health systems.

The NHSS National Quality Standards and criteria define goals that are common to different types of services. This means that although the evidence for a criterion may vary, consideration of evidence can be made in the context of organisational structure, geography, patient type and services.

The National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), all PHAs, and other ‘Health Service Organisations’ are encouraged to undertake and present additional activities that respond to the criteria within each standard, and is regarded as total quality rather than a checklist of compliance and provides direction for improvement activities and for achieving better practice.

Each element identifies what is in place to at least fulfil the minimum requirements of the criterion at a certain rating level.

CORE FUNCTIONS

The **Core Functions** in the NHSS 2nd edition sets out against the National Quality Standards enable the NRSTTH, all PHAs, and other ‘Health Service Organisations’ to achieve quality improvements against the standards and address:

1. Organisational Governance and Health System Strengthening for a PHA – healthcare organisations,
2. Quality & Safety in Integrated Health Service Delivery,
3. Essential Public Health Services,
4. Essential Clinical Care.

ACCREDITATION AND ACCOUNTABILITIES

NATIONAL DEPARTMENT OF HEALTH

The Minister for Health drives policy direction and investment around safety and quality: NHP 2022-2030 KRA 5: Health system strengthening: building strong resilient health systems: leadership, governance and management is strengthened at all levels of the health system;

- Strategy: the NRSTTH, all individual PHAs, and other ‘Health Service Organisations’ undertake mandatory continuous quality assessment & improvement accreditation program that supports excellence in consumer / patient care & health service delivery;
- The Minister for Health receives information from Boards of Governance (i.e. NRSTTH, individual PHAs, and other ‘Health Service Organisations’ about performance against the National Quality Standards for health service organisations healthcare services.

- ▲ The National Health Board awards the accreditation status i.e., awarded, retaining accreditation or withdrawal;

- The NDOH receives data on outcomes of assessments of the health services provided by the NRSTTH, individual PHAs and other 'Health Service Organisations' and responds to any emerging issues;
- The NDOH provides the 'Secretariat' to facilitate the assessment of the NRSTTH, individual PHAs, and other 'Health Service Organisations', against the National Quality Standards, provides the logistic arrangements, manages financing arrangement for organisational-wide accreditation surveys, registers accredited 'peer surveyors', tracks timeframes for 4yr accreditation cycles; receives, summarises & monitors action plans and risks, related to NRSTTH, individual PHAs, and other 'Health Service Organisations';
- NDOH Secretariate collects and analyses feedback on the lessons learnt through accreditation as an essential part of quality improvement and uses feedback to maintain and update the Standards and identifies areas where the NRSTTH, an individual PHA, or other 'Health Service Organisation, may require additional support or tools;
- The NDOH provides advice and discusses any issues that the NRSTTH, individual PHA, or other 'Health Service Organisation' may have.....
- Through coordination of the NHSS, NDOH, the NRSTTH, individual PHAs, and other 'Health Service Organisations' will:
 - Develop and maintain the National Quality Standards for healthcare services in PNG;
 - Undertake ongoing liaison on opportunities to improve the Standards and the accreditation system;
 - Report to the Ministry of Health through the Secretary for Health annually on safety and quality.

NATIONAL REFERRAL SPECIALIST TERTIARY AND TEACHING HOSPITAL (NRSTTH), PROVINCIAL HEALTH AUTHORITIES (PHAS) AND OTHER 'HEALTH SERVICE ORGANISATIONS'

- The NRSTTH, all individual PHAs and other 'Health Service Organisations' are held accountable to improve the quality of health care and protect the public from harm through the accreditation process;
- The NRSTTH, all individual PHAs and other 'Health Service Organisations' undertake a formal 4 Phase Accreditation process, 4-year accreditation cycle linked to their Board of Governance Performance Framework and provide progress on actions remediated; the NRSTTH, each individual PHA and other 'Health Service Organisation' nominates a main contact to be the organisation's Quality Coordinator;
- The NRSTTH, individual PHAs and other 'Health Service Organisations' implement the actions required to meet the National Quality Standards; and
- The selection of trained accredited peer surveyors is from across the NRSTTH, individual PHAs and other 'Health Service Organisations' (i.e government, church health, NGOs and private health service providers), (released from duties) and available for organisational wide accreditation surveys.
- The nominated Quality Coordinators within the NRSTTH, each individual PHA and other 'Health Service Organisation' are also trained accredited officers.

NATIONAL QUALITY STANDARDS

The objectives of the **NQS Accreditation Survey** are to:

- Assess the quality of services provided by the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations' against the national quality standards.
- Provide a team of Surveyors to serve as consultants and educators for health services.
- Make recommendations for self-improvement.

The Set of National Quality Standards are as follows:

FUNCTION 1:

ORGANISATIONAL GOVERNANCE (NHP KRA 5): implement a Corporate & Clinical (Public Health) Governance framework throughout the whole of the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations'.

1. Leadership and Management, Governance for Safety & Quality in Health Services:
2. Corporate Governance Systems & Safety (OH&S - Mandatory standard),
3. Clinical & Public Health Systems & Safety (Mandatory standard).

HEALTH SYSTEM STRENGTHENING (NHP KRA 3 + 5) implementation of all resource standards through the whole of the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations':

4. Health Workforce Capacity and Performance,
5. Training, Ongoing Professional Development,
6. Essential Medical Supplies Procurement and Supply Chain Management Systems,
7. Health Infrastructure & Improve Facility - Environmental Management (Fire Safety - Mandatory Standard),
8. Financial Resource Management,
9. Health Information & Information Communication Technology for Decision Making.

FUNCTION 2

QUALITY AND SAFETY IN INTEGRATED HEALTH SERVICE DELIVERY: implement a health service delivery system through, the not who, but the whole of the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations'.

10. Health Service Planning:
 - Planning & Implementation, working collaboratively to plan, monitor, measure, analyse, and communicate about the health status of the population;
11. Partnerships & Collaboration,
 - Partnerships & Service Agreements,
 - Strengthen, support and mobilize communities and partnerships to improve health. Partnerships work with local and district level governments and health partners to provide assistance, capacity building and resources for local efforts and mobilize partnerships to collectively identify and solve health problems;
12. Universal Health Coverage (UHC) principles and drive for implementation of NHSS by the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations';
13. Preparedness for Disease Outbreaks and Emerging Population Threats Including Pandemics (Mandatory standard);
14. Formalised Patient Referral Pathways;
15. Supportive Supervision;
16. Infection Prevention and Control (Mandatory Standards);
17. Waste Management (Mandatory Standards).

FUNCTION 3

Target Health Priorities are integrated into quality and safety in health service delivery, and addressed in essential public health services and clinical care.

FUNCTION 3: ESSENTIAL PUBLIC HEALTH SERVICES (EPHS): focus on public health quality standards for implementation through the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations' and include:

18. Integrated evidence-based interventions into public health practices in all its work. Evidence-based interventions and Clinical, PHC & Public Health intersectional and connectivity lead to more successful programs and policies, greater workforce productivity and better use of resources;
19. Essential public health service standards implementation leads to evidence-based prevention approaches to wellness and the prevention of chronic diseases and injury and builds capacity to address leading causes of death and disability;
20. Comprehensive health promotion & disease prevention as an integral part of all healthcare services; and
21. Respond to public health challenges through environmental health programs.

FUNCTION 4

ESSENTIAL CLINICAL CARE: focus on implementation of essential clinical care across levels of health services throughout the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations':

- 22. Appropriate & Effectiveness of Care & Health Services;
- 23. Provision and Continuity of Patient Care (Mandatory Standard);
- 24. Medication Management;
- 25. Surgical Safety;
- 26. Blood Products.

ESSENTIAL CLINICAL CARE IN PARTNERSHIP WITH PUBLIC HEALTH

The supervisory checklist and scorecards will support the core functions and standards.

Clinical Care Services includes:

- Emergency Services
- Ambulatory Care Services
- Critical Care Services
- Integrated Clinical Care and Public Health in Care of Patients with Non-Communicable Diseases and Communicable Diseases
- Surgical Services inc: Oral Health Program
- Maternal and Newborn Care (O&G) Services
- Newborn and Child Health Services
- Oncology and Palliative Care Services
- Family Support Services
- Pre and Post-Operative Care
- Anaesthetics and Pain Management
- Operating Theatre and Endoscopy Procedures

Allied Health and Diagnostic Care Services:

- Rehabilitation: Patient Mobility, i.e. Amputations, post CVA, Respiratory, Brain Injury
- Physiotherapy
- Nutrition Dietetics
- Orthotics Prosthetics
- Occupational, Audiology, Speech Therapies
- Social Work: Facilitate the welfare of communities, individuals, families, and groups
- Pastoral / Spiritual Care

Radiology & Medical Imaging:

- Radiology, Ultrasound
- Pathology
- Public Health Laboratory
- Mortuary
- Blood Bank Transfusion Service linked to Blood Product Standard

Health Facility Environmental Management:

- Biomedical Engineering: Medical Equipment linked to Health Infrastructure Management Standard
- Sterilisation Services
- Medical Records Management inc: Clerical support record-keeping
- Pharmacy Drugs and Medical supplies, Consumables, Medical Gasses, Cold Chain & Vaccines linked to Medical Supplies Procurement and Supply Chain Management System Standard and Medication Management Standards
- Portage Services.

HEALTH ACCREDITATION

ACCREDITATION

Accreditation is the status obtained by the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations' after a successful peer survey external evaluation facilitated by the NDOH to assess whether the NRSTTH, individual PHA, or other 'Health Service Organisation' meets applicable predetermined and published National Standards.

Accreditation is one aspect of performance measurement / requirements set within the NHP & NHSS.

The focus of accreditation programs is to provide a framework for continuous improvement. It is about establishing a structure and processes that allow quality and safety to proliferate throughout a health service organisation.

The National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations' that successfully achieve an accreditation status endorsed by the National Health Board and awarded full four years accreditation receive country-wide recognition by the Minister for Health and official acknowledgement of the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations' commitment to continuous quality improvement.

Both the Minister for Health and the Health Secretary sign and present a '*Certificate of Accreditation*' promoting quality. The NRSTTH, each individual PHA, and other 'Health Service Organisations' celebrate the achievements of accreditation.

The National Quality Standards and Accreditation process is a continuous quality improvement program so it is not expected that all standards are completely achieved over time. There will always be room for continuing improvement and surveyors provide guidance.

Standards relating to the safety of patients and staff (mandatory standards) need to be satisfactorily achieved for formal accreditation to be awarded.

Depending on the nature of the risk, the NRSTTH, individual PHA and/or other 'Health Service Organisation' may receive a 'conditional accreditation' for one year, or not be awarded an accreditation at all.

The survey team makes a recommendation on accreditation status and recommends a final accreditation status.

All information about the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations' is classified as 'confidential' and bound by agreement with the surveyors (as per NHSS Vol 2 2011).

Both NDOH Secretariat and surveyors sign a confidentiality statement on appointment as peer surveyors.

The NDOH, Boards of Governance for NRSTTH, individual PHAs and other 'Health Service Organisations' may publish the accreditation status of the organisation, for example whether or not the organisation is accredited and the period of that accreditation.

In addition, the NDOH may also publish non-identified aggregated data from surveys for the purpose of informing the health care sector and the public and encouraging improvement in the quality of care.

BENEFITS OF ACCREDITATION

Means all people working in the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations' are:

- ▲ empowered to improve the delivery of service and quality care
- ▲ provided with benchmarks for quality outcomes
- ▲ equipped with a rigorous approach to continuing improvement
- ▲ given a comprehensive report for continuous improvement.

For the PNG Government it provides:

- ▲ assurance that the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations' are operating according to National Quality Standards
- ▲ confidence that the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations' are patient and community focussed
- ▲ evidence of quality outcomes
- ▲ confidence in the way the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations' minimises and manage risks and achieve better value for the money spent on health.

THE ACCREDITATION CYCLE

Accreditation against the National Quality Standards requires the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations' to participate in a four-year cycle of events, with one activity to be completed during each year of the cycle.

A four (4) year cycle comprises of:

1. Self-Assessment by the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations': a. at a minimum from within each district and each level of care & priority essential services;
2. Organisational Wide Accreditation Survey of the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations': a. by accredited trained peer surveyors;
3. Ongoing Self-Assessment: a. progress on action taken towards addressing recommendations from the previous survey that are implemented organisation-wide across ALL health services within the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations';
4. Periodic Review: a. Survey of mandatory criteria and progress on recommendations.

SELF-ASSESSMENTS

The National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), each individual PHA, and other 'Health Service Organisations' need to assess their achievements and outcomes regularly to improve performance. Using a standardised assessment tool, the NRSTTH, each individual PHA, and other 'Health Service Organisation' can compare and continually update and review their performance against standards.

The Self-Assessment provides a 'gap analysis' that the NRSTTH, individual PHAs and/or other 'Health Service Organisation' can use as a basis for service improvement planning. Self-assessments review progress against the recommendations of a previous survey.

This allows the NRSTTH, individual PHAs, and other 'Health Service Organisations' to continue to improve in the areas that were identified during the previous survey as needing development.

In addition, the NRSTTH, individual PHAs, and other 'Health Service Organisation' submits an updated Risk Register of key organisational risks and a copy of the Quality Improvement Plan to their individual Board of Governance, and to the NDOH.

The **Risk Register** includes a risk rating and action plan for the key risks to the NRSTTH, individual PHA, and other 'Health Service Organisation' as a whole.

This demonstrates that the major risk areas to the NRSTTH, individual PHA, or other 'Health Service Organisation' is being proactively managed and that this is contributing to the overall safety and to quality improvement.

The **Quality Improvement Plan (QIP)** documents the key areas for improvement within the NRSTTH, individual PHA, or other 'Health Service Organisation'.

The QIP outlines how the need was identified, how the NRSTTH, individual PHA, and other 'Health Service Organisation' intends to carry out the improvements, the timeframes and designated responsibilities, reporting arrangements including responsibilities for this, and the evaluation processes including the expected outcomes / improvements.

ASSESSMENT PHASES OF ACCREDITATION

Phase 1 Self-Assessment for NRSTTH, PHA, or other 'Health Service Organisation'

- 1st self-assessment against all criteria.
- Provide progress on action taken towards addressing recommendations from the previous survey to **CEO – Board of Governance** of NRSTTH, PHA, or other 'Health Service Organisation'
- Submit register of key risk (risk register).

Phase 2 Organisational Wide Accreditation Survey of NRSTTH, PHA, or other 'Health Service Organisation'

- 3 months prior to formal survey, provide to **NDOH** self-assessment against all criteria and progress on action taken towards addressing recommendations from previous survey and the Quality Improvement Plan.
- Full risk register provided to the surveyors at the time of survey.
- All criteria are surveyed and progress on recommendations from the previous survey is

Phase 3 Self-Assessment

- Provide progress on action taken towards addressing recommendations from the previous survey.
- Submit register of key NRSTTH, PHA or other 'Health Service Organisation' risks (risk register).
- Submit Quality Improvement Plan.

Phase 4 Periodic Review

- 3 months prior to a periodic review key NRSTTH, PHA or other 'Health Service Organisations' provide to **NDOH** a self-assessment against all mandatory criteria and progress on action taken towards addressing recommendations from the previous survey.
- The Quality Improvement Plan is updated.
- Full risk register is provided to surveyors at survey.
- Mandatory criteria are surveyed and progress on recommendations from previous

SELF-ASSESSMENT

A report is provided for each Self-Assessment by an internal Peer Survey Team of the NRSTTH, individual PHA, or other 'Health Service Organisation' and is designed to give helpful suggestions on quality improvement and how to enhance or strengthen the survey process in preparation for the next onsite survey or review.

The NRSTTH, individual PHAs, or other 'Health Service Organisations' that already have quality systems in place and an infrastructure for regularly reviewing care and service may simply need to review and / or modify their current approaches to incorporate the National Quality Standards and Accreditation process.

The NRSTTH, individual PHAs, or any other 'Health Service Organisations' new to a quality accreditation program need to establish structures and systems using the National Quality Standards as a guide. Once a Self-Assessment is completed it can then be reviewed and becomes a living document.

The National Quality Standards are designed to be incorporated into everyday practice. The certified trained peer surveyors are available to answer any questions and provide additional support as required.

PHA – ORGANISATION WIDE SURVEY (OWS)

An Organisation Wide Survey is conducted by selected Peer Surveyors from across the NRSTTH, individual PHAs, or other 'Health Service Organisations' (i.e. Church Health Services, NGOs, Private health Sector).

They are experienced and trained peers who provide feedback on overall performance against the National Quality Standards.

The survey involves several components including:

- ▲ presentations made by staff to demonstrate their achievements and improvements in all major areas of operations within the NRSTTH, individual PHAs or other 'Health Service Organisations';
- ▲ verification by surveyors of the NRSTTH, individual PHA, or other 'Health Service Organisations' achievement's in relation to National Quality Standards;
- ▲ summation of the surveyors' findings, recommendations and guidance for future action;
- ▲ The first Organisation Wide Survey generally occurs approximately 12 months after the 1st self-assessment and a survey varies from 5 – 10 days depending on the size and scope.

Risk ratings are allocated at the OWS every four (4) years.

Compliance with all the National Quality Standards for Health Services in Papua New Guinea, by the NRSTTH, all PHAs and other 'Health Service Organisation' is assessed by a full survey team. The step which the NRSTTH, individual PHA or other 'Health Service Organisation' takes when seeking formal accreditation.

A self-assessment is usually carried out by members of the NRSTTH, individual PHA or other 'HealthService Organisation' who have been trained in the National Quality Standards and survey methods.

They write a report, including recommendations, to help the NRSTTH, individual PHA or other 'Health Service Organisation' prepare for an external accreditation survey by a team of independent Surveyors. The internal survey helps the organisation to assess its own 'strengths' and 'weaknesses'.

A 'baseline survey' is one carried out by members of the NRSTTH, individual PHA, or other 'Health Service Organisation'. Again, they prepare a report to help the organisation prepare for a formal external accreditation survey.

An OWS is a survey by an external team of trained Surveyors who conduct the assessments and provide a report to the Board of Governance and CEO, of the NRSTTH, individual PHA, or other 'Health Service Organisation' on outcomes of health services assessments against the NQS and criteria. The survey team examines documents, facilities, equipment and services and interviews members of staff and patients to assess compliance with the National Quality Standards.

The external survey team usually comprises a Chief Executive Officer (CEO) and representatives from Nursing, Medical Services, and Public Health services, and can include a Finance and Administration, Provincial / District Program or Health Manager, the survey team considers the need for a Clinical Surveyor to assist with surveying clinical units. These peer representatives spend from two (2) to five (5) days on site in a number of health service delivery settings.

At the conclusion of the survey, there is a summation conference to discuss the survey team's findings with the respective Board of Governance, CEO, and Senior Executive Management team, of the NRSTTH, individual PHA, or other 'Health Service Organisation'.

The Surveyors complete their report, which contains an assessment of each service for each level of care and commendations and recommendations. The report remains the property of the NRSTTH, individual PHA, or other 'Health Service Organisation' with a copy provided to the NDOH Executive Manager, Medical Standards Division.

The final phase of the process is the signing and presentation of the '*Certificate of Accreditation*' to the NRSTTH, individual PHA, or other 'Health Service Organisation' concerned, by both the Minister for Health and the Secretary of Health.

The Certificate can also be delegated for presentation by a senior member of the NDOH at ceremony within the NRSTTH, individual PHA, or other 'Health Service Organisation'.

Accreditation is also announced at the annual National Health Conference.

PERIODIC REVIEW

A Periodic Review is an onsite survey that is undertaken approximately two (2) years after the NRSTTH, individual PHA, or other 'Health Service Organisations' – Organisation Wide Survey in order to further facilitate ongoing health service improvement and maintenance of achievements.

This review includes:

- ⬆ action taken in response to recommendations from the previous survey
- ⬆ organisational performance in relation to the Mandatory Criteria.

SURVEY REPORTS

Following Organisation Wide Survey and Periodic Review, a summary of the surveyors' findings is compiled to provide the NRSTTH, individual PHA, or other 'Health Service Organisation' with:

- ⬆ a detailed assessment of its current performance against the National Quality Standards and criteria
- ⬆ identification of areas where performance is satisfactory or where further action is required
- ⬆ positive comments in areas of high achievement
- ⬆ suggestions and recommendations for further improvements.

RESPONSE TO RECOMMENDATIONS FROM ORGANISATION WIDE SURVEY OR PERIODIC REVIEW

The National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), individual PHAs, or any other 'Health Service Organisation' is required to maintain the Quality Improvement Plan and planned action (including target dates) in response to the recommendations from the last survey event.

This is reviewed at each Self-Assessment and by the surveyors at the next survey event.

ACCREDITATION CITERION RATING

Criteria have been developed around a rating scale designed to measure and support continuous quality improvement in key areas of a health service operations.

Each of the criteria has four possible levels of achievement:

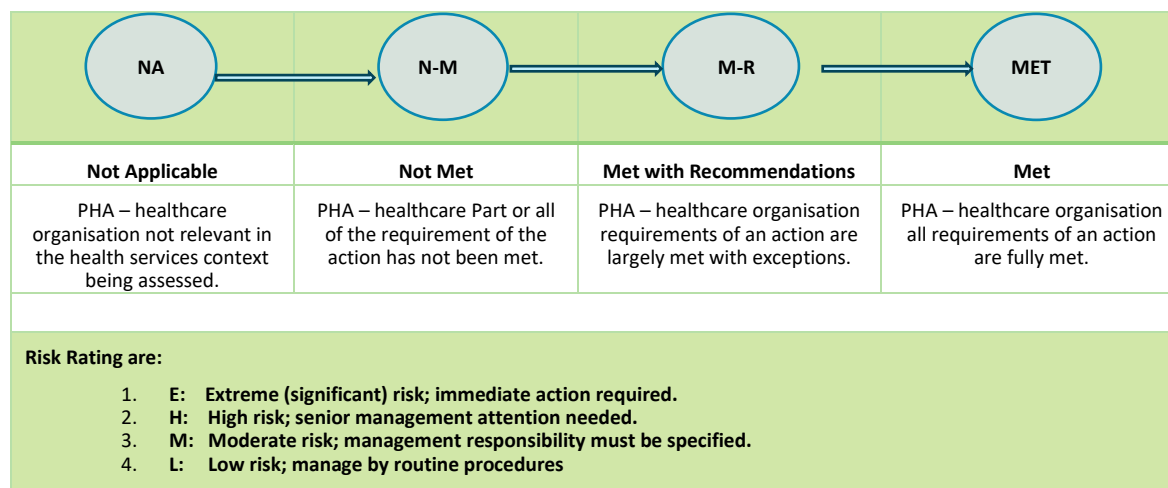
- I. Not Applicable,
- II. No Met,
- III. Met with Recommendations,
- IV. Met.

A risk rating is also applied to actions where recommendations are given to show the level of risk associated with the particular action. A risk comment is also included if the risk is rated greater than low. Risk rating are

- a) Extreme Risk,
- b) High Risk,
- c) Moderate Risk
- d) Low Risk.

Surveyors use the elements in each of the criteria to rate the level of the PHA – healthcare organisation's achievement. The elements are not meant to be an audit checklist; they describe the current practices that contribute to the achievement of each level.

While the manner in which the elements are implemented and the evidence presented may differ between PHAs - healthcare organisations, each is able to demonstrate current practices addressing the intent of the elements and the evidence.



The **criterion rating format** assists the NRSTTH, individual PHA, or other 'Health Service Organisation' to better understand how the elements can support continuous quality improvement.

- **NA:** There are a few criteria and elements in the NQS that may not be applicable within the NRSTTH, individual PHA, or other 'Health Service Organisation'. Criteria may be described as not applicable (NA) when the topic of the criterion would never occur or would never have the potential to occur.

NB: All NQS Functions and criteria will be considered applicable to the NRSTTH, all PHAs, and other 'Health Service Organisations', except where it has been formally agreed that they are not applicable.

It is accepted that in some settings or levels of care there may be some criteria which are only partially relevant, but in these cases the criteria will still be applicable for onsite survey purposes.

- **Not Met – Met with recommendations – Met:** The goal is to determine whether the systems that the NRSTTH, individual PHA or other 'Health Service Organisation' has implemented, work effectively and whether there is *potential to improve*, and perhaps to give insight into *how this might be achieved*.

NB: Evaluation is judging the value of something by gathering valid information about it in a systematic way and by making a comparison; its purpose is to help to decide what to do, to justify earlier actions or to contribute to scientific knowledge.

Evaluation of a process or system involves a review of performance measures and other qualitative data, including the opinions of those directly involved in, or affected by, a process. It may include comparisons against earlier times, other departments or against consumers / patients. Evidence of data collection, its evaluation, and action taken, or reasons for inaction, would be expected to determine the rating.

MANDATORY CRITERIA

A mandatory criterion is one where it is considered that without evaluation, the quality of care or the safety of people within the NRSTTH, individual PHA, or other 'Health Service Organisation' could be at risk.

Mandatory criteria are those where a rating of 'Met' - to gain or maintain accreditation.

The seven mandatory requirements are:

1. **Corporate Governance Systems and Safety, including OH&S**
2. **Clinical Public Health Governance Systems and Safety**
3. **Preparedness for Disease Outbreaks and Emerging Population Threats**
4. **Environmental Management: Health Infrastructure, Equipment and Facility Management – eg: Fire Safety**
5. **Provincial and Continuity of Care**
6. **Infection Prevention and Control**
7. **Waste Management.**

RISK MANAGEMENT, PROCEDURE AND REGISTER

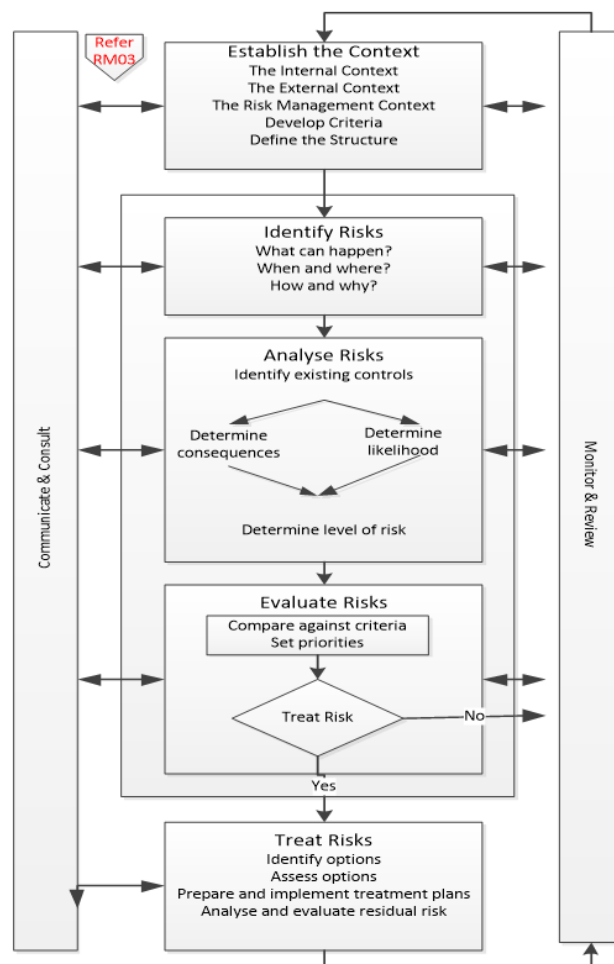
The NRSTTH, individual PHAs, and other 'Health Service Organizations' must recognise that safety and quality management should be focussed on managing opportunity as well as losses. It recognises that a structured approach to the management of safety and quality encourages the identification of greater opportunities for continuous improvement through innovation. The NRSTTH, individual PHAs, and other 'Health Service Organisations' must have an integrated approach to safety and quality management.

Safety and Quality is an integral component of both Corporate and Clinical Public Health Governance Framework which outlines the strategies for systematically identifying opportunities for continuous improvement as well as managing risk. Risk management is integrated into a PHA normal business. Key risks are communicated across the organisation and strategies implemented to mitigate those risks. These are regularly monitored and reviewed by the PHA CEO and SEMT, Clinicians, Managers and the PHA Board along with opportunities for improvement.

Risk Management Actions: The NRSTTH, individual PHA, and other 'Health Service Organisation': Identifies and documents organisational risks; Uses clinical and other data collections to support risk assessments; Acts to reduce risks; Regularly reviews and acts to improve the effectiveness of the risk management system; Reports on risks to the health workforce and consumers; Plans for, and manages, internal and external emergencies and disasters.

The expected outcome of an effective safety and quality management program is to ensure:

- Maximising of opportunities and minimisation of losses.
- Management perspective and encourage initiative and pro-active behaviour in relation to risk management.
- Greater openness in decision-making and improve communication by engaging key stakeholders and consumers in the review and decision-making process.
- Concise summary of the major risks affecting the organisation are presented to the CEO and SEMT of the NRSTTH, individual PHA, or other 'Health Service Organisation' and appropriate resources are directed towards areas of high risk.
- Effective and systematic approach which enables management to focus on areas of risk in their operations.
- All staff understands how risk is assessed, through identification, analysis and evaluation.
- Essentially risk management is an ongoing activity and recognising and reducing risk should be integral to the organisational culture.



The diagram above, describes the process adopted in identifying and eliminating risk throughout an organisation.

RESPONSIBILITIES

It is the responsibility of all workplace participants to identify, report and minimise risk throughout the NRSTTH, individual PHAs, or other 'Health Service Organisations'. It is the responsibility of each individual to report accidents and incidents when they occur.

It is the responsibility of management at all levels, within the NRSTTH, individual PHAs, and other 'Health Service Organisations', to ensure accidents and incidents are reported and escalated: (where they become aware of accidents/incidents personally or through employee/agent/contractor report and notification).

To meet this commitment, risk is to become every health employee's business. All employees are required to be responsible and accountable for managing risk in so far as is reasonably practicable within their area of responsibility. The following procedure should be followed by all staff to identify, control and manage risks within the NRSTTH, individual PHA, or other 'Health Service Organisation':

Establish the Context: The criteria against which risk will be evaluated should be established and the structure of the risk analysis defined in the following steps:

- **Establish the external context** – Defining the external environment in which XXXX practices operates and the relationships with external parties.
- **Establish the internal context** – Ensuring there is a sound knowledge of the way XXXX and its practices function internally. Specifically, this includes understanding: the culture; the structure; the stakeholders; the capabilities; the resource base.
- **Establish the risk management context** – This ensures that risk management is developed within the context of XXXX and each of its practices' boundaries. Understanding where everybody fits in and the roles and responsibilities.
- **Establish the risk criteria** – Defining how risks will be identified, ranked and treated both at a local practice and a national/corporate level.
- **Establish the structure for the rest of the process** – Ensuring the framework for managing risk is logical, comprehensible and manageable.

Identify Risks: Risks are identified at each setting or practice and documented in a Risk Register. Risks may also be identified through incident reports (including near misses), improvement logs, workplace inspections, changes to work processes, procedure, protocols, and/or purchases of new equipment.

Where an immediate risk is identified, this should be reported (as above) and escalated to the appropriate Committee / Delegated person to be reviewed and actioned. Where the risk is 'endorsed' it shall be added to the appropriate Risk Register at the next appropriate forum.

All employees of the NRSTTH, individual PHAs, and other 'Health Service Organisations' have an obligation to identify risks and ensure that all the appropriate people within the organisation are made aware of them. Once identified, preventive measures can be taken and put in place to control the risks (so far as is reasonably practicable).

There are three levels of risk registers:

1. **NRSTTH and PHA Hospital Units Risk Register:** where the Hospital Unit risk register manages local level risks but also informs the organisational risk register
2. **Health Facility Risk Register:** where the Health Facility risk register manages local level risks but also informs the organisational risk register
3. **Organisational Risk Register** – separated into:
 - a. Corporate Risk Register
 - b. Clinical Public Health Risk Register
 4. Board of Governance Risk Register
 - a. NRSTTH, PHA, or other 'Health Service Organisation' Risk Register.

Determining Risk: In determining risks some important areas to consider are:

- Global risks that would or could impact practices
- Risks within and outside span of control
- Risks associated with activities conducted outside health facility sites
- Risks associated with outsourced or contracted services
- Risks associated with the use by external organisations for goods and services.

Sign Posts: In identifying risks some important areas to consider are:

- Lessons from the past
- Results of audits or physical inspections
- Records of prior losses, (i.e. claims, financial or property losses, data/record loss, incidents and safety reports).
- Training provided to staff
- Judgements – consensus, speculative/conjecture, intuition
- Results of benchmarking for perceived performance deficiencies
- Gap analysis – distinguishing between existing practice and planned objectives

Cause and Effect of Risk: It is critical in the assessment of risk, that the following are considered:

- The cause of an exposure (i.e. failure of..., lack of..., loss of..., injury to....)
- The effect of the exposure - the effects may include clinical impact on patients, financial impact, impact on staff, and other stakeholders, impact on reputation and probity, impact on operational management and impact on the delivery of services to patients.

Inspection Program: At XXXX we assess and reduce risk through regular review and inspection programs.

These programs are designed to be flexible in nature and include:

- Routine inspection of all risks
- Routine inspection / assessment of a particular or area of risk
- Specific inspections resulting from recommendations, patient or referrer complaints, reports or advice from staff, users, stakeholders and others. This includes investigations and/or inspections recommended by the provincial – district health and safety committee or management committee
- Inspections as a result of incidents or accidents
- Inspections as a result of audits.

Analyse & Evaluate Risks: The risk analysis and evaluation processes consider the range of potential consequences and how likely they are to occur. Consequence and likelihood are combined to produce an

estimated level of risk. Consequence and likelihood are described in the XXXX Risk Matrix to enable an accurate estimate of the level of risk.

Treat Risks: The Risk Matrix descriptors identify the level of action required associated with the level of risk. For higher priority risks, a specific risk management plans will be developed including funding considerations. Lower priority risks may be accepted (including by elimination of the risk, or failing this, minimization) and monitored.

Monitor and Review: This is the oversight and review of the risk management system and any changes that might affect it. Monitoring and reviewing occurs concurrently throughout the risk management process and is defined in the PHA Committee Structure as per the Clinical Public Health Governance Framework or Corporate Governance Standards.

Ongoing Review: Although monitoring and review is a key element of risk management, pre-defining review times ensures continuity. The following review times will occur:

Review Times	Actions
Immediate	Escalation of risks which have substantial impact to the business or a patient(s) and meet determined escalation tolerance levels to the relevant Board of Governance Committee, or the CEO & SEM Executive of the NRSTTH, individual PHA, or other 'Health Service Organisation'. Evaluation will be monitored as per the action plan.
Quarterly	Monitor and review all risks through the following processes: Hospital Unit Risk Register: The authorised person will review the status of practice wide risks and actions with key staff in their respective setting and document the risk actions taken in that quarter. Any new or changed risks will be identified and escalated as necessary. The Manager of each setting will report any new / key risks to the Executive Manager of the Specialty and key stakeholders. The Unit or Health Facility Managers will monitor the progression of the Risk Register. Organisational Risk Register: The SEMT Executive Manager, in consultation with the Managers will review the specialty register and document the risk actions taken in that quarter. Any new or changed risks will be identified and escalated as necessary. The Risk Register will be updated and the SEMT will monitor the progression of risk actions and risk reduction. Board of Governance Risk Register: The Board of Governance or (Risk Management subcommittee) is provided with a current Risk Register. Particular emphasis is to be given to Extreme and High-Risk Issues and any respective outstanding Proposed Corrective Actions. The Risk Register will be updated and the Executive Management Team will report to the Board through the CEO the progression of risk actions and risk reduction.
Annually	The risk management process is reviewed by the relevant Board of Governance of the NRSTTH, PHA or other 'Health Service Organisation' for efficiency and effectiveness. Including: Reviewing the risk contexts for each Hospital Dept – Unit – Health Facility and Audit of the Risk Register by the external Auditor as required.

The annual review timeline is established to enable appropriate identification of new / existing risks that may impact on budget and operational planning. By establishing a defined timeline, practices can effectively manage risk and plan risk control strategies in a more effective manner. When risks are identified, risk mitigation strategies must be initiated until more robust controls can be established.

Evaluation: 100% of incidents will be risk rated; A risk register exists for all levels of the PHA – healthcare organisation; Risk Register review occurs as per the reporting timelines.

Annexures: Risk Matrix, severity rating and reporting.

QUALITY IMPROVEMENT PLAN (QIP)

The goal of an effective Quality Improvement Plan (QIP) is in improving performance of healthcare systems of the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), individual PHAs, and other 'Health Service Organisations'.

Quality Improvement Plans (QIPs) serve as the road maps for the quality management effort of the NRSTTH, individual PHA and/or other 'Health Service Organisation'.

The aim of a Quality Improvement Plan is to help workers within the NRSTTH, individual PHAs, and other 'Health Service Organisations' to self-assess performance in delivering quality health care, and to plan future improvements.

A QIP must:

- include an assessment of the health services and public health programs and practices at the hospital or health facility against the NHSS National Quality Standard and National Regulations, and
- identify any areas for improvement.

Using the quality improvement cycle can be helpful in encouraging teams to reflect on practice and to identify ways of improving service performance.

Stages of quality improvement:

- plan – do – check – act;
- what to do?
- Who is responsible?"
- When to report back.

See Annexures: for Quality Improvement Plan template and example.

ACCREDITATION SURVEYORS

The role of the Accreditation Surveyor requires:

- Team work;
- Diplomacy and tact;
- Objectivity and impartiality;
- Expertise; and
- Knowledge.

Surveyors are ambassadors for the Papua New Guinea Health Services National Quality Standards process. The credibility of the National Quality Standards depends on the conduct and performance of Accreditation Surveyors.

Another important responsibility of the Surveyor is to serve as a sounding board and working partner to his or her surveyor colleagues.

Therefore, Surveyors must be able to evaluate and offer comment on other areas being surveyed besides their own specialty areas and must be active 'champions within the NRSTTH, an individual PHA, or other 'Health Service Organisation' and its health services.

Surveyors are consultants and educators during the formal survey. They offer advice to help identify and resolve quality-related difficulties.

They offer informal on-site education to help prevent future problems developing and to set health services on the right path towards achieving the National Quality Standards.

Accreditation Surveyors must also continually assess their ability to carry out their role of evaluator in an environment where increasing complexity and diversity is the norm and change is continual.

BENEFITS OF BEING AN ACCREDITATION SURVEYOR

The benefits of being an Accreditation Surveyor are as follows:

- The opportunity to keep knowledge up-to-date through studying the National Quality Standards and contact with other Surveyors and Peers across all health services and health programs being surveyed.
- The opportunity to observe firsthand the different organisation structures and processes of health services within the NRSTTH, individual PHAs, and other 'Health Service Organisation'.
- The observation of good initiatives which may enhance services in their own healthcare organisations.
- The capacity to tap into a network of health services providers.
- The educational experience of sharing and attempting to resolve problems and difficulties facing facilities and programs in the delivery of health care country-wide.
- The professionally rewarding experience of being part of a survey team, with members of other professions, and providing health services with a report, drawing on practical experiences in the provision of health care in PNG.

- The privileged experience of being invited to enter any area of a facility or health service/program being surveyed, to talk to staff, patients or visitors and to review the way in which services are organised and provided.
- The opportunity to see how people work in health services of all sizes and types and to help the different groups understand their counterparts in other areas, i.e. people from small rural and remote health services understanding their counterparts in large teaching health services and vice versa.
- The benefit of being continually reminded of the performance that the Surveyor's own organisation should be achieving and observing how different facilities meet the National Quality Standards within their resources.

ELIGIBILITY TO BE AN ACCREDITATION SURVEYOR

Any Chief Executive Officer (CEOs), Directors of Curative / Medical Services or Director of Nursing Services, Districts Public health, Health Manager, Provincial / District Program Manager, Quality Coordinators, Senior Officers of NGOs, are eligible to train to be a National Quality Standards Accreditation Surveyor.

Chief Executive Officers of NRSTTH, individual PHAs, and other 'Health Service Organisations' may also nominate any other suitably senior and experienced member of his or her staff to be a Surveyor, i.e. Director of Finance and Administration, Infection Control Officer, Chief Pharmacist, Senior Clinician, etc.

Suitably senior and experienced officers of the National Department of Health (NDoH) may also be nominated for Surveyor training.

The National Department of Health, Medical Standards Division, works collaboratively with all CEOs about the suitability of any nominees who are not currently employed in a health service before confirming that they will be trained as Surveyors.

RESPONSIBILITIES OF AN ACCREDITATION SURVEYOR

- Undertake an observation survey.
- Commitment to the implementation of the National Quality Standards.
- A commitment of two surveys per year if required.
- Approval from own NRSTTH, PHA or other 'Health Service Organisation' employer to participate in surveys.
- Understanding and applying the National Quality Standards in their own organisation, in a variety of health services.
- Using up-to-date knowledge of governance, leadership, management and continuous quality improvement principles.
- Participation in Surveyor updates.
- Agreement to be involved in continuing assessment of Surveyor's performance.
- Agreement to submit survey reports at the completion of the survey.

- Agreement to be involved with education and advisory activities or National Quality Standards development.
- Agreement to be a coordinating Surveyor once adequately experienced.

ACCREDITATION SURVEYING TASK

The primary responsibility of the Surveyor is to conduct surveys of health services delivered within the National Referral Specialist Tertiary and Teaching Hospital (NRSTTH), individual PHAs, and other 'Health Service Organisations' and to provide comprehensive, relevant and timely reports of the surveys so that the relevant health service has a prompt indication of the outcome of its survey.

Surveyors are responsible for keeping themselves up-to-date with National Quality Standards, surveying techniques and procedures. At orientation, all new Surveyors receive a copy of A Guide to National Quality Standards for Health Services in Papua New Guinea – Health Service Accreditation Program, which details policies and procedures for the conduct of surveys and the survey process.

Health professionals are selected as Surveyors on the following requirements:

- Understanding of the National Quality Standards and the philosophy that underpins them.
- Suitability based on present employment responsibilities.
- Commitment to the National Quality Standards accreditation process.
- Competence to write reports.
- Time management.
- Acceptance of criticism and self-correction.
- Understanding of quality activities and quality in health care.
- Credibility among his or her peers.
- Ability to work in a team.
- Interpersonal skills.

ACCREDITATION SURVEYOR EDUCATION

There are four main components in Surveyor education. They include:

- An orientation workshop for all new Surveyors;
- Observation surveys undertaken by new Surveyors;
- Annual Surveyor updates; and
- Continuing education.

All Surveyors must attend an annual update and, if unable to do so, they attend the annual Surveyor orientation. Surveyors are not eligible to undertake surveys unless they have attended an orientation, an update or undertaken an observation survey within the last 24 months.

SURVEYOR ORIENTATION

A workshop for new Surveyors is usually held once each year.

The aims of Surveyor orientation are to:

- Familiarise Surveyors with the National Quality Standards;
- Provide Surveyors with the knowledge, skills and attitudes that they need; and
- Provide Surveyors with a supervised opportunity to observe and participate in a survey.

At the completion of the workshop, Surveyors are able to:

- Explain the purpose of the National Quality Standards for Health Services in Papua New Guinea; the benefits to the health service of being surveyed; and the effect of a National Quality Standards Accreditation Survey on the community, the Boards of Governance and providers of health services.
- Describe the National Quality Standards and the intent of them, their application within health services and health centres and the National Quality Standards Accreditation Survey process.
- Describe the role and functions of the Surveyor.
- Survey a health service or health centre effectively.

OBSERVATION SURVEYS

Trainee Surveyors are invited to participate in a survey as an observer. The role of an observer is explained in more detail.

ANNUAL SURVEYOR UPDATES

Surveyor updates is usually held once each year.

CONTINUING EDUCATION

From time-to-time the National Department of Health, Medical Standards Division, coordinates Surveyors, the opportunity to participate in continuing education activities relevant to the National Quality Standards Accreditation process.

GRATUITIES

The NRSTTH, individual PHAs, and other 'Health Service Organisations' must extend reasonable hospitality to the survey team. Surveyors must not accept a gift of any significant cash value from the NRSTTH, individual PHAs, or other 'Health Service Organisations' which are subject to a health services survey.

THE COORDINATING SURVEYOR (TEAM LEADER)

As well as ensuring the smooth coordination and management of the survey and the production of the Survey Report, the Coordinating Surveyor is the spokesperson for the surveying team.

In this important role, the Coordinating Surveyor makes the initial contact with the NRSTTH, individual PHA, or other 'Health Service Organisation', establishes leadership of the survey itself, and throughout the survey is alert to any potential problems and defuses difficult situations before they escalate.

On rare occasions, problems arise during the survey. The Team Leader - Coordinating Surveyor acts responsibly to resolve any dispute and maintain the good standing of the survey team and process. Advice may be sought from the Preceptors in regard to any issue, before or during the survey or at the report writing stage. Advice may also be sought from Preceptors on any aspect of the survey, interpretation of the National Quality Standards or report writing.

The Coordinating Surveyor is also free to seek assistance from the National Department of Health, Medical Standards Division at any time during, before or after the survey.

PRECEPTORS

A 'Preceptor' is a teacher or instructor. Senior Surveyors (nominated at the Surveyor update meeting) serve as Preceptors for the National Quality Standards Accreditation Survey Program in the initial stages.

A Preceptor may:

- Accompany a survey team undertaking a survey in some circumstances;
- Be contacted by Surveyors at any time to discuss aspects of the program and any issues relating to the National Quality Standards; and
- Provide advice to the National Quality Standards and Accreditation Secretariat within the National Department of Health's Medical Standards Division, on administrative matters relating to accreditation.

NEW SURVEYOR OBSERVER

New Surveyors may be attached to a survey team as observers for practical experience after they have participated in a new Surveyor training program. The observation survey is designed to enable new Surveyors to observe how compliance with the National Quality Standards is assessed during the course of a survey.

Permission for an Observer to be present on a survey is arranged by the Secretariat with the relevant CEO and the Coordinating Surveyor prior to the survey. In general, only one Observer participates in a survey.

The Coordinating Surveyor may delegate a component of the survey to an Observer, such as writing a section of the report, but an Observer may participate in interviews only under the direct supervision of an experienced Surveyor.

The new Surveyor's activities as an Observer are determined by the Coordinating Surveyor within the following framework. The Observer is expected to:

- Share in the preparation of a plan for surveying the services within the NRSTTH, individual PHA, or other 'Health Service Organisation'.
- Report to other members of the survey team on observations made during the survey.
- Attend the whole survey including both the pre-survey conference and the summation conference. The Observer may be asked to take an active part in the summation conference.
- Formulate an opinion on the level of compliance of one or more services with the Standards and report to the survey team. The objective here is both to add to the education of the observer but also to improve inter-rater reliability by providing an independent score for one or more services.
- Write a portion of the Survey Report for submission to the responsible Surveyor.
- Be evaluated by the survey team.

CONFLICT OF INTEREST FOR SURVEYORS

Surveyors do not survey where there is a conflict of interest.

Should a conflict of interest arise a Surveyor informs the Secretariat promptly.

Cases of a conflict of interest include:

- A healthcare organisation where they have previously worked in a senior post.
- A healthcare organisation with which they have a close professional or personal relationship.
- A healthcare organisation they have surveyed within the last five years.
- A healthcare organisation with which the Surveyor's affiliated organisation is in a competitive situation.
- Any other situation where a conflict of interest would exist, i.e. prospective employment, employment as consultant within five years.

The NRSTTH, individual PHA, or other 'Health Service Organisation' may appeal against a particular Surveyor being included in the survey team to the the National Department of Health, Medical Standards Division if it believes that a conflict of interest exists.

A change of Surveyor may be made at the discretion of the NDOH Secretariat.

ANNEXURES

ANNEX 1:
NATIONAL QUALITY STANDARDS FOR HEALTH SERVICES IN PAPUA NEW GUINEA

See separate documents

ANNEX 2:
SURVEYOR CERTIFICATE

ANNEX 3:
CERTIFICATE OF ACCREDITATION

ANNEX 4: RISK MATRIX, SEVERITY RATING AND REPORTING

RISK MATRIX, SEVERITY RATING AND REPORTING REQUIREMENTS

Risks – Likelihood of Occurring						
Descriptor	Description					
Almost certain	The incident will most probably occur under most circumstances > 75% probability of occurring in the next 12 months					
Likely	The incident will most probably occur under most circumstances 51-75% probability of occurring in the next 12 months					
Possible	The incident will most possibly occur under most circumstances 21-50% probability of occurring in the next 12 months					
Unlikely	The incident is unlikely to occur under most circumstances 5-20% probability of occurring in the next 12 months					
Rare	The incident will only occur under the most exceptional circumstances < 5% probability of occurring in the next 12 months					

LIKELIHOOD		CONSEQUENCES				
		Insignificant	Minor	Moderate	Major	Extreme
	Almost Certain	Medium	High	High	Very High	Very High
	Likely	Medium	Medium	High	High	Very High
	Possible	Low	Medium	High	High	High
	Unlikely	Low	Low	Medium	Medium	High
	Rare	Low	Low	Medium	Medium	High

SEVERITY RATING

Categories of Risk	Insignificant (Inc. Near Miss)	Minor	Moderate	Major	Extreme
Financial	An event with an ongoing negative impact of <5% to net assets. - Minimal impact to financial reporting.	An event with an ongoing negative impact of >5%<10% to net assets. - Immaterial impact in financial reporting.	An event with an ongoing negative impact of >10%<15% to net assets. - Significant error/issue in financial reporting.	An event with an ongoing negative impact of >15%<20% to net assets. - Material error in financial reporting causing reputational damage and regulator intervention.	An event with an ongoing negative impact of >20% to net assets. - Fundamental error/going concern issue in financial reporting.
Strategic	No impact on strategy but local/state actions required.	Minor impact on strategy with negative local/state media coverage.	Adversely impacts attainment of strategic outcomes with negative local/state media coverage.	Adversely impacts attainment of strategic outcomes with short term negative nationwide media coverage.	Inability to execute company strategy with long term negative nationwide reputational damage.
Operations	Consequences can be absorbed via normal management.	Consequence can be absorbed through strong management of action/rectification plans.	Significant event managed through action/rectification plans.	A critical event that can be endured if strongly managed.	Significant disruption and cost to operations over an extended period of the year.
Compliance	Minor breaches by individual employee /contractors that can be remedied quickly and locally. Full accreditation ongoing with recommendations to be actioned	No penalty but additional time and resources required to remedy the situation. Full accreditation ongoing with high priority recommendations to be actioned	No penalties but significant disruption to operations resolved within 1 month. Partial accreditation impact as unmet mandatory criteria needs to be addressed immediately	Penalties and disruption to operations resolved within 3 months. Loss of accreditation Threat to organisational viability – inc. health fund negotiations etc.	Significant fines or loss of licences resulting in loss of service delivery over an extended period. Inability to source insurance / service providers impacting on organisational viability

Categories of Risk	Insignificant (Inc. Near Miss)	Minor	Moderate	Major	Extreme
Employees	Minimal injury requiring no/minimal intervention or treatment No time off work	First aid treatment only with no lost time or restricted duties	Medical expenses, lost time or restricted duties or injury / illness for 1 or more employees / volunteers Psychological treatment required for < 3 months	Permanent injury to employee/volunteer, Hospitalisation of 1-2 employees/volunteers, or lost time or restricted duty or illness for 2 or more employees/volunteers Pending or actual Work Cover prosecution, or Threatened or actual physical or verbal assault of employees/ volunteers requiring external or police intervention	Workplace incident leading to death, including suicide Hospitalisation of 3 or more employees / volunteers
Patient	Patients with: No injury; or No increase in level or care length of stay	Patients requiring increase in medical attention including: First Aid treatment Review and evaluation (incl additional observations) Additional investigations - Referral to another clinician - but no admission to another facility.	Patient requiring increased level of medical attention that is unrelated to the natural course of the illness and differing from the expected outcome of patient management Increased length of stay in facility Cancellation or delay in planned procedure/treatment	Patients with severe health crisis i.e. permanent loss of function unrelated to the natural course of the illness and differing from the expected outcome of patient management Patient at significant risk due to being absent against medical advice Patient suffering significant harm as a result of the incident Threatened or actual physical or verbal assault of patient requiring external intervention (Police etc)	Death of a patient which is unrelated to the natural course of the illness and differing from the expected outcome of patient management plan. A Sentinel Event Hospitalisation of 3 or more persons for 3 or more days Suspected suicide/homicide
Categories of Risk	Insignificant (Inc. Near Miss)	Minor	Moderate	Major	Extreme
Visitors Contractors	1. No injuries and no treatment required 2. Reportable near miss 3. Contracted service delivery maintained	1. Temporary injury or illness 2. First aid required 3. Hospital admission LOS of ≤ 5 days 4. Report as required to relevant external bodies, Inability to access contractor with some impact on service	1. Medical treatment required 2. Hospital admission LOS 6-20 days 3. Report as required to relevant external bodies 4. Executive involvement in accident / incident investigation 5. Inability to source replacement contractor resulting in temporary reduction of service provided 6. Intervention for emotional/ spiritual/physical impact	1. Permanent injury or illness 2. Extensive injuries 3. Hospital admission with LOS > 20 days 4. Report as required to relevant external bodies 5. Discontinuation of service impacting on organisational objectives 6. Intervention and referral for emotional / spiritual / physical impact	1. Death or unexpected death, &/or Severe permanent disability or injury 2. Report as required to relevant external bodies 3. Loss of contracted service provider impacting on core business 4. Discontinuation of multiple services due to inability to source contractor 5. Intervention for emotional / spiritual / physical impact to extended family

Facility Security Environment	Services: No loss of service (0-1 hours) <u>Environment:</u> Nuisance releases	Services: Reduced efficiency or disruption to service functionality <u>Environment:</u> Off-site release contained without outside assistance	Services: Disruption to users due to service/agency problems <u>Environment:</u> Off-site release contained with outside assistance required	Services: Major loss of agency / service to users >6r & <12hr <u>Environment:</u> Off-site release with no detrimental effects or fire	Services: Complete loss of service and/or output > 12 hours <u>Environment:</u> Toxic release off-site with detrimental effect. Emergency requiring evacuation (external intervention) &/or EPA intervention
Reputation Complaints	1. Peripheral element of treatment or service suboptimal 2. Informal complaint received	1. Overall treatment or service sub-optimal 2. Formal complaint received 3. Local resolution required 4. Single failure to meet internal standards 5. Minor implications for patient safety if unresolved 6. Reduced performance rating if unresolved	1. Treatment or service has significantly reduced effectiveness 2. Significant formal complaint received 3. Local resolution (with potential to go to independent review) 4. Repeated failure to meet internal standards 5. Major patient safety implications if findings are not acted on	1. Non-compliance with national standards with significant risk to patients if unresolved 2. Multiple complaints received / independent review required 3. Low performance rating	1. Incident leading to totally unacceptable level or quality of treatment/service 2. Gross failure of patient safety if findings not acted on 3. Inquest / ombudsman inquiry 4. Gross failure to meet national standards

SEVERITY REPORTING RESPONSE

Categories of Risk	Insignificant (Inc. Near Miss)	Minor	Moderate	Major	Extreme
Incident Severity Rating Response	Report to: Line Manager	Report to: Line Manager Department Head or Health Facility Manager	Report to: Line Manager Dept Head or health Facility Manager SEMT relevant Executive Directory Quality Coordinator/ Manager	Immediately contact and report to: Line Manager Dept Head or health Facility Manager SEMT relevant Executive Directory Quality Coordinator/ Manager	Immediately contact and report to: Line Manager Dept Head or health Facility Manager SEMT relevant Executive Director Quality Coordinator/ Manager CEO

RISK REGISTER

Name of Dept – Unit – Health Facility	Location	NRSTTH / PHA / other 'Health Service Organisation'	Responsible Person	Role Performed

Risk Category	Location	Identified Risk	Risk rating*	Registered Date	Risk Controls (strategies currently in place)	Risk Treatment Actions – (further actions required to mitigate / control risk)	Due Date	Person Responsible
Strategic								
Operational								
• Clinical								
• Non-Clinical								
Reporting								
Compliance								

* For risk rating please insert likelihood rating, consequence rating and risk rating e.g. rare x minor = Low

Broad categories of risk	Strategic – Operational – Reporting - Compliance
Reporting of the Risk Register	- The Risk Register will be reported to the Quality Coordinator and the CGAC and SEMT of the respective NRSTTH, individual PHA, or other 'Health Service Organisation' on a quarterly basis, as a standing agenda item - Medical risks will be reported to the Director Curative Health quarterly - The Risk Register will inform the NRSTTH, individual PHA, or other 'Health Service Organisation' about the Organisational Risk Profile and be reported to the SEMT and PHA Board quarterly
Associated documents	- PHA Clinical Public Health Governance Framework - Risk Matrix - Incident, Near Miss, Categories of incidents and Management – Critical Incidents register within the Clinical Public Health Governance Framework and Manual

ANNEX 5: QUALITY IMPROVEMENT PLAN AND EXAMPLE

Quality Improvement Plan - EXAMPLE This is an illustrative example to assist in identifying the key components of an effective Quality Improvement Plan (QIP).

ACTION PLAN															
Fields >	#	Status	Improvement	Delivery Date	Rationale	Current > Target	Source	Risk	Priority	Lead	Action	Due Date	Owner	Progress	Commentary
Description >	1	Is the improvement opportunity in progress, completed, or under evaluation?	State specifically the improvement - what does the PHA want to achieve?	The date the improvement is expected to be fully delivered and implemented	Explain why this improvement is important and why now	Define the current state - what is the baseline performance data? Define the future state - what performance target would identify the improvement as a success? For example: an increase in compliance rates, or an evaluation of the completed guidelines, etc.	State how the improvement was identified. For example: via an internal audit, gap analysis, assessment report, incidents, complaint, etc.	What is the risk if this improvement is not implemented? Use a risk matrix to identify the relevant risk rating	Prioritise the improvement according to the risk rating and rationale You can select whatever priority ranking you prefer	Identify one (1) position of the person or group responsible and accountable for designing, implementing, measuring and evaluating the improvement.	Describe the actions the PHA will take to implement the improvement. For example: modification of specific protocols or processes, provision of training, etc.	Document the date of expected completion	Identify one (1) position of the person responsible and accountable for designing, implementing, measuring and evaluating the improvement.	Note if the objective has been completed or evaluated	Important notes for management of the improvement and related activities
Example >	2	In Progress	Improve hand hygiene compliance for all healthcare professionals (visiting and staffed) throughout all inpatient and outpatient and urban-rural health facilities	01-Sep-20	Hand hygiene is an essential component of PHA's infection control plan to avoid harm to our patients and reduce avoidable readmissions or extended length of stay	Increase compliance from 65% to 85%	Internal Audit - scheduled hand hygiene observational rounds	High	High	PHA Infection Prevention Control Advisory Committee	Review internal audit results and identify where missed 'HH Moments' were missed	01-May-2022	Internal Audit Manager	In Progress	Review underway and report to be provided at next meeting
											Develop mandatory in service - online refresher courses	10-Jun-2022	PHA Health Training & Resource Centre Manager	In Progress	
											Develop onsite training for new hires and visiting doctors	01-Jul-2022		Not Started	

											Conduct 'in ward – health facility' education sessions on hand hygiene importance and practices	01-Aug-2022	Clinical Educator	In Progress	
											Launch Hand Hygiene competition between wards and department – health facilities to reward performance	01-Jun-2022	SEMT – Directors Curative & Nursing	In Progress	
											Evaluate effectiveness of improvement actions and implement updates accordingly	15-Aug-2022	Infection Prevention & Control Advisory Committee	Not Started	

Quality Improvement Plan

Health Facility – Hospital Department – Unit Name	
Current as of:	10-Apr-2022

ACTION PLAN

#	Status	Improvement	Delivery Date	Rationale	Current > Target	Source	Risk	Priority	Lead	Action	Due Date	Owner	Progress	Commentary